

Standard Operating Procedure

Safe Clinical Procedure Verification

«Hospital Name» — Supports COP.7.c, COP.7.d, COP.7.e (Stop-Work)

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» verifies patient, procedure, and site before every clinical procedure — diagnostic, therapeutic, or supportive — performed outside the operating theatre.

2. Scope

Applies to every diagnostic, therapeutic, or supportive procedure performed at the bedside, in a procedure room, or in an outpatient setting — central line insertion, bronchoscopy, biopsy, lumbar puncture, and similar.

3. Responsibility

Privileged Performing Clinician: carries final verification responsibility for the procedure.

Assisting Staff: confirms identifiers independently before the procedure starts.

4. Procedure

1. Confirm the procedure is clinically indicated and consistent with standard treatment guidelines or sound clinical practice before proceeding.
2. Confirm you are privileged for this specific procedure before performing, ordering, planning, or assisting in it.
3. Assess and record the patient's vitals before every procedure.
4. Verify the patient using two identifiers, one of which is the unique identification number generated at registration.
5. Confirm the correct site where applicable, involving the patient or relative in marking whenever possible.
6. Obtain informed consent, signed by the person performing the procedure or a doctor from the treating team. Where a trainee performs the procedure, state this in the consent and name the supervising doctor.
7. Monitor the patient during and after an invasive procedure — at minimum pulse, blood pressure, and respiratory rate, plus any clinically indicated parameters.
8. Document the procedure fully: name, who performed it, key steps, key findings, and post-procedure care, plus date, time, and signature.
9. In a genuine emergency where full verification cannot be completed, document the specific step that was skipped and why, in the medical record.

5. Stop-Work Authority

Do not begin a non-emergency procedure without completed two-identifier verification and, where applicable, site confirmation.

Do not proceed if the performer is not confirmed as privileged for that specific procedure.

Stop-work applies to elective or non-emergency procedures — it does not block genuine emergency intervention, which proceeds with the exception documented afterward.

Any staff member present may raise a verification gap before the procedure starts; this is not a disciplinary matter.

6. Records

Supported by Records 5, 6, 7, 8, 9, 10, and 11 of the COP.7 Required Records pack.

7. Related Documents

- HCO_COP_7_v2_REWRITE_DRAFT — COP.7 policy
- SOP — Surgical Safety Checklist (COP.14, for OT-based procedures)

8. Review

Reviewed annually, and immediately after any wrong-patient, wrong-procedure, or wrong-site near-miss.

9. References

- World Health Organization. (2008). WHO Surgical Safety Checklist (first edition). <https://iris.who.int/items/08d5cc88-060c-4f03-8a94-330c095d49bc> — the verification principles in this SOP (correct patient, procedure, site) are adapted from the WHO checklist model for use outside the operating theatre.